	Manual Of Operations	
	COP	Guidelines For Antenatal Services
Hope Hospitals	Version: 3.0	Page: 1
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1. Objective

To provide comprehensive, evidence-based, safe, and respectful antenatal care to all pregnant women visiting Hope Hospital, ensuring maternal and fetal well-being through the continuum of pregnancy.

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2. Scope

Applies to:

- **All pregnant patients visiting OPD**
- **Nurses, RMOs, Obstetricians, Lab, Sonography, and Pharmacy**
- **First to third trimester care**

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3. First Antenatal Visit (Booking Visit)

🗓 **Ideally between 6–12 weeks of gestation.**

Checklist:

- **ANC Registration: ANC card issued**
- **History Taking: Obstetric, medical, surgical, family history**
- **Vitals & BMI**
- **Obstetric Exam: Per abdomen/pelvic if needed**
- **Baseline Investigations:**
  - **CBC, Blood Group & Rh**
  - **HIV, HBsAg, VDRL**
  - **RBS, Urine Routine/Microscopy**
  - **TSH, HCV (optional)**

📋 ***Documentation:*** **Booking form, antenatal register, patient file**

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4. Antenatal Schedule (As per GCP/NHM/WHO)

Visit No.	Timing	Assessments
1st	6–12 weeks	Booking labs, dating scan, risk stratification
2nd	14–18 wks	Anomaly scan requisition, Iron-Calcium started
3rd	20–24 wks	Anomaly scan review, Fetal growth monitoring
4th	28–30 wks	GTT (Glucose Tolerance Test), Td1, Hb review
5th	32–34 wks	Growth scan, FHR auscultation, BP, edema check

6th	**36–38 wks**	**Delivery plan discussion, internal exam**
7th+	**>38 wks**	**Weekly monitoring until delivery**

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5. Clinical Monitoring Each Visit

- **BP & Weight**
- **Fundal Height**
- **Fetal Heart Sound (FHS) – from 16 weeks**
- **Fetal Movement Count – from 24 weeks**
- **Proteinuria check if hypertensive or edematous**
- **Counseling on nutrition, danger signs, delivery prep**

📝 ***All findings recorded in ANC card***

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6. High-Risk Identification

Flagged if:

- **Anemia (<9 g/dL)**
- **PIH, GDM, twin pregnancy**
- **BOH, Rh-negative**
- **Teenage or elderly gravida**

🟥 **High-Risk Sticker applied on file; specialist follow-up as per SOP**

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7. Immunization

- **Tetanus + Diphtheria (Td):**
  - **1st Dose: ~16–20 weeks**
  - **2nd Dose: 4–6 weeks after 1st (at least 1 month before EDD)**

- *If previously vaccinated in last 3 yrs, only 1 booster needed*

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8. Antenatal Supplements

Drug	Start From	Dose
Folic Acid	At confirmation	400 mcg OD
Iron + Folic Acid (IFA)	14–16 wks	100 mg elemental iron + 500 mcg folic acid
Calcium	From 20 wks	500 mg twice daily
Deworming	~16 weeks (single dose albendazole)	

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9. Investigations & Imaging

Trimester	Investigations	Imaging
1st	Routine labs, TSH, Hb, HIV, VDRL, RBS	Dating Scan
2nd	GCT, Hb, Urine, LFT/RFT (if PIH)	Anomaly Scan (18–22 wks)
3rd	Hb, GTT (if not done), USG if IUGR/PIH	Growth Scan, BPP

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10. Health Education & Counseling

- **Diet during pregnancy (caloric intake, iron-rich foods)**
- **Danger signs: bleeding, pain, headache, swelling, decreased fetal movement**
- **Birth preparedness plan**
- **Breastfeeding counseling (from 32 weeks onward)**
- **Postnatal contraception options**

📄 Education Leaflet provided at 20 weeks

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11. Patient Rights & Communication (PRE)

- **Informed about findings in local language**
- **Privacy and dignity ensured during all checkups**
- **Consent taken for any invasive procedure (e.g., Internal exam, USG)**
- **Grievance redressal contact listed on ANC file**

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12. Record Keeping

✅ **Mandatory records include:**

- **ANC Card (Triplicate)**
- **Lab Reports**
- **USG Reports**
- **Consent Forms**
- **High-Risk Register**
- **Immunization Register**

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13. Quality Indicators for ANC Services

Indicator	Target
≥4 ANC Visits per registered woman	>90%
Anemia detection and treatment rate	>95%
ANC documentation completeness	>95%
High-risk case timely referral rate	100%

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14. Training & Review

- Monthly ANC case audit
- Quarterly refresher training for nurses & RMOs on:
  - Danger signs
  - Safe handling of ANC equipment
  - High-risk case identification